



[w]Health Healthcare and Life Sciences Index

From awareness to accountability
in women's health

Kearney Foresight
Health Institute



KEARNEY FORESIGHT

Health Institute

The Kearney Health Institute, part of the Kearney Foresight network, convenes leaders from healthcare, life sciences, and beyond to provide foresight and drive collective action that impacts health outcomes worldwide.

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Foreword

The health of women reflects the health of society. As highlighted in the open letter released by Kearney, decades of underinvestment, bias, and neglect have left women with poorer outcomes across nearly every measure of health. Life expectancy, disease diagnosis, access to care, and participation in research all reveal the same pattern of inequity. Closing this gap requires coordinated action across sectors. It demands new systems designed around the realities of women’s biology, lived experience, and economic participation.

Every organization has a role in that transformation. Many recognize the urgency but struggle to know where to begin. Progress often stalls not because of a lack of intent, but because companies lack the frameworks, benchmarks, and data needed to guide effective action. Without shared standards, individual efforts remain fragmented and the potential for collective impact is lost.

To address this challenge, Kearney, in collaboration with the United Nations Population Fund (UNFPA)-led Equity 2030 Alliance and the Healthcare Businesswomen’s Association, developed the [w]Health Index. The Index establishes the first cross-sector baseline for women’s health equity, offering transparency, accountability, and a road map for measurable progress. It assesses organizations across six levers of change: public health, medical education, research and development, care pathways, investment, and data. By turning evidence into practical guidance, the Index enables each organization to understand where it stands, identify gaps, and learn from leading practices that are redefining what it means to advance the health of women.

The case for action is clear. Improving the health of women is both a moral and an economic imperative. Healthy women strengthen families, drive productivity, and contribute to more resilient economies. Yet persistent inequities cost trillions each year through preventable illness and deaths, lost productivity, and reduced participation in the workforce. Addressing these gaps is not only a public health priority but a catalyst for stronger economies and more resilient societies.

The [w]Health Index serves as both a benchmark and a call to collective action. It translates commitment into measurable action and helps ensure that the health of women becomes central to the health and progress of the world.



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Executive summary

The case for action in women's health is no longer in question. Persistent inequities across research, care delivery, access, and outcomes continue to undermine health systems, constrain innovation, and impose significant economic costs. Women live longer than men, yet spend more years in poor health, experience delayed diagnoses, and face care pathways that too often fail to reflect their biology or lived experience. Closing these gaps is both a moral imperative and a strategic opportunity to strengthen health systems, improve productivity, and build more resilient economies.

The [w]Health Healthcare and Life Sciences Index reveals that momentum is building across the healthcare ecosystem, but progress remains uneven and fragmented. While approximately half of organizations have taken steps to address the gender health gap, only 38 percent of healthcare and life sciences organizations demonstrate mature, system-level action. Progress is strongest in advocacy and education, but lags significantly in inclusive research, women-centric care design, data integration, and investment. These gaps persist across sectors, company sizes, and regions, underscoring that the challenge is systemic rather than geographic.

The Index makes clear that the next phase of progress requires a shift from awareness to accountability. Advancing women's health cannot rely on isolated initiatives or goodwill alone. It demands that sex- and gender-specific considerations are embedded into core decision-making across the value chain, from R&D and clinical trial design to care pathways, reimbursement, data systems, and capital allocation. Organizations that have moved further along this path show stronger governance, clearer ownership, and greater integration of women's health into strategy and performance management.

Sector-specific insights highlight both opportunity and responsibility. Consumer health companies lead in maturity, driven by data, design, and direct engagement with women, but must now translate scale into system impact. Pharma and MedTech have made progress in policy and education, yet remain constrained by limited inclusion in research, validation, and care design. Providers and payors play a critical role in translating innovation into equitable access, but too often struggle to align benefits, coverage, and delivery with women's health needs. Investors are emerging as catalysts for change, but gender-lens capital remains underutilized despite strong fundamentals and growing interest.

Across all sectors, three priorities are essential. First, embed sex and gender into core operations so that equity is designed into innovation, not retrofitted. Second, strengthen data and accountability by linking sex-disaggregated insights to measurable KPIs, leadership incentives, and transparent reporting. Third, align access, investment, and incentives to ensure that innovation reaches women across life stages and geographies.

The [w]Health Index provides a practical framework to support this transition. By establishing a shared baseline, benchmarking progress across sectors, and identifying concrete actions, it enables organizations to move beyond intent toward coordinated, measurable change. The question facing leaders is no longer whether women's health should be a priority, but how decisively they are prepared to act. Those that lead in the next 12 months will help redefine what effective, inclusive, and future-ready healthcare looks like.

Introduction

Why we need to redesign healthcare with women in mind

For many years, inequities at the heart of healthcare have gone unaddressed, with potentially devastating consequences for half the global population. Decades of underinvestment, inadequate awareness, and systemic bias have left women more likely to experience misdiagnosis, dismissal of symptoms, and poorer long-term outcomes. Women live on average [six years longer than men](#) yet spend more of those years in poor health. For example, [four out of five pain medications](#) have historically been tested only in male animals, leaving critical gaps in understanding women's responses. Cardiovascular disease is another stark example: women are [twice as likely to die](#) following a heart attack as men, in part due to underdiagnosis and treatment gaps.

The implications extend beyond public health to directly affect businesses and economies. Health inequities for women result in higher absenteeism, lost productivity, and increased healthcare costs. Conditions such as endometriosis and menopause can reduce workplace participation, with women reporting significant weekly hours lost due to unmanaged symptoms. Chronic conditions, when untreated, lead to costly hospitalizations and preventable complications that strain health systems. For investors and innovators, these inequities represent missed opportunities: multibillion-dollar markets like menopause care, autoimmune disorders, and cardiovascular conditions remain underfunded despite clear demand and growth potential. According to the World Economic Forum, closing the gap is estimated to [add \\$1 trillion to global GDP by 2040](#)—not only improving quality of life for women but also building stronger and more resilient economies.

Traditionally, women's health has been defined too narrowly, focused almost exclusively on reproductive and sexual health. While these areas remain essential, they reflect only a fraction of women's overall health needs. The greater burden lies in conditions that [disproportionately or differently](#) affect women, ranging from autoimmune and neurodegenerative diseases to osteoporosis, depression, and cardiovascular disease. Momentum to broaden this definition is accelerating, driven by high-profile initiatives and the growth of FemTech, which are reshaping prevention, diagnosis, and treatment. Yet achieving real change requires more than innovation. It demands individual accountability, collective progress, and systemic transformation across the entire healthcare ecosystem to ensure that the health of women is recognized and prioritized as fundamental to human and economic well-being.

The [w]Health Index was designed to accelerate this change. By benchmarking maturity across the six levers of action, the Index provides both a baseline and a road map. It helps organizations assess where they stand, learn from peers, and take concrete steps toward closing the gender health gap. In short, it turns the case for action into a framework for accountability and progress.

The [w]Health Index: purpose and methodology

The [w]Health Index is a new tool designed to help organizations across sectors take meaningful action to advance women's health. Because the challenges are interconnected, spanning workplaces, communities, and healthcare systems, companies must have the ability and responsibility to act. The Index drives accountability by assessing current activities, enabling comparison with peers, and highlighting best practices. It establishes a baseline and industry benchmarks so that collectively progress can be measured, gaps identified, and action toward better health and business outcomes accelerated.

To build a comprehensive view of how the industry is performing, structured surveys were combined with in-depth interviews across North America, Europe, and Asia Pacific. The analysis includes respondents from hundreds of organizations spanning six sectors that together represent the healthcare ecosystem: pharma, MedTech, consumer health, payors, providers, and investors.

The Index looks at progress through six levers for change:

- Shape public health agenda through increased advocacy and awareness, with governments putting women's health on—and higher up—the political agenda and targeting sources of stigma and bias.
- Overhaul medical education by expanding both national school and higher education curriculums to adequately cover women's health topics and eliminate sources of bias for future health professionals and patients.
- Raise the stakes on R&D by increasing the volume of clinical and policy research trials on women's health conditions and ensuring fair gender representation in all trials.
- Build women-centric integrated care pathways that are accessible and easy to navigate, covering all patient touchpoints and providing women with more control over their own health.
- Get serious about data by ensuring gender-specific data sets are collected, regulated, analyzed, used, and shared in a responsible way across the healthcare ecosystem, advancing health outcomes not just for individuals but in the general population.
- Accelerate investment by boosting funding for academic research, product development, and consumer health solutions with active input from across the ecosystem.

Each organization was mapped against these levers using four maturity stages. *Exploring* indicates that women's health is just beginning to enter the agenda; data may be limited and conversations still early. *Advancing* reflects growing momentum, with pilot programs or benefits in place, though often fragmented. *Transforming* signals that women's health has become a strategic priority, embedded in cross-functional teams, KPIs, and outcome measurement. At the highest level, *leading* organizations set the standard by influencing policy, investing at scale, and creating platforms that extend their impact beyond their own walls.

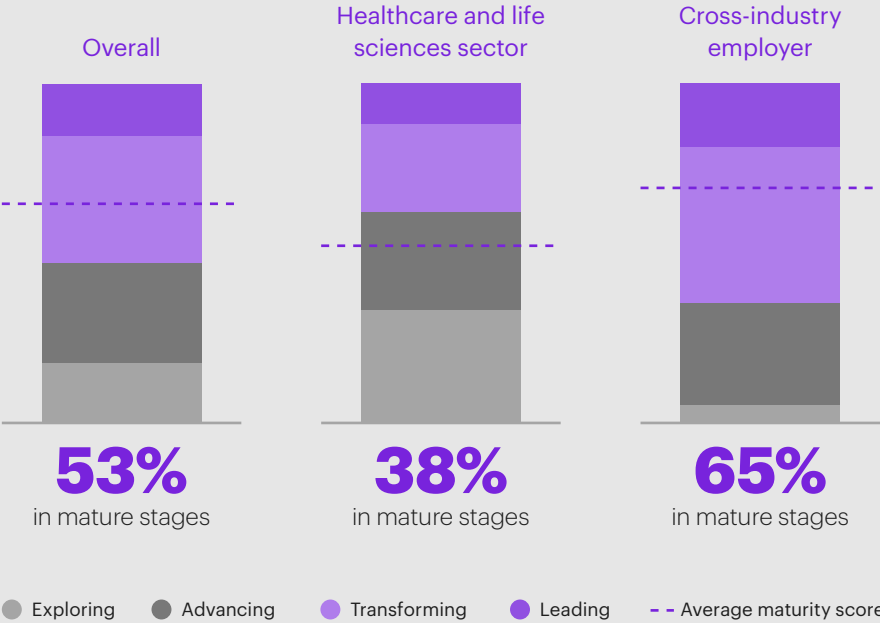
Key findings by sector

Momentum is growing, but maturity remains uneven across the ecosystem

Results reveal clear momentum but uneven progress. Across the ecosystem, just 38 percent of organizations have reached mature stages of action (see figure 1 on page 6). Larger healthcare players tend to show stronger performance, while regional variation remains limited (see figure 2 on page 6). This maturity is strongest in advocacy and education, while progress in research, data, and women-centric care continues to lag behind (see figure 3 on page 7). Across sectors, consumer health companies are furthest ahead, driven by their consumer-centric approach, and demonstrate advances in data and R&D (see figure 4 on page 8). Pharma and MedTech show progress in policy and education but lag in women-centric care and inclusive research. Providers and payors are making headway in awareness and benefit design but remain constrained by affordability and access challenges, and investors are only beginning to apply a gender lens to portfolios.

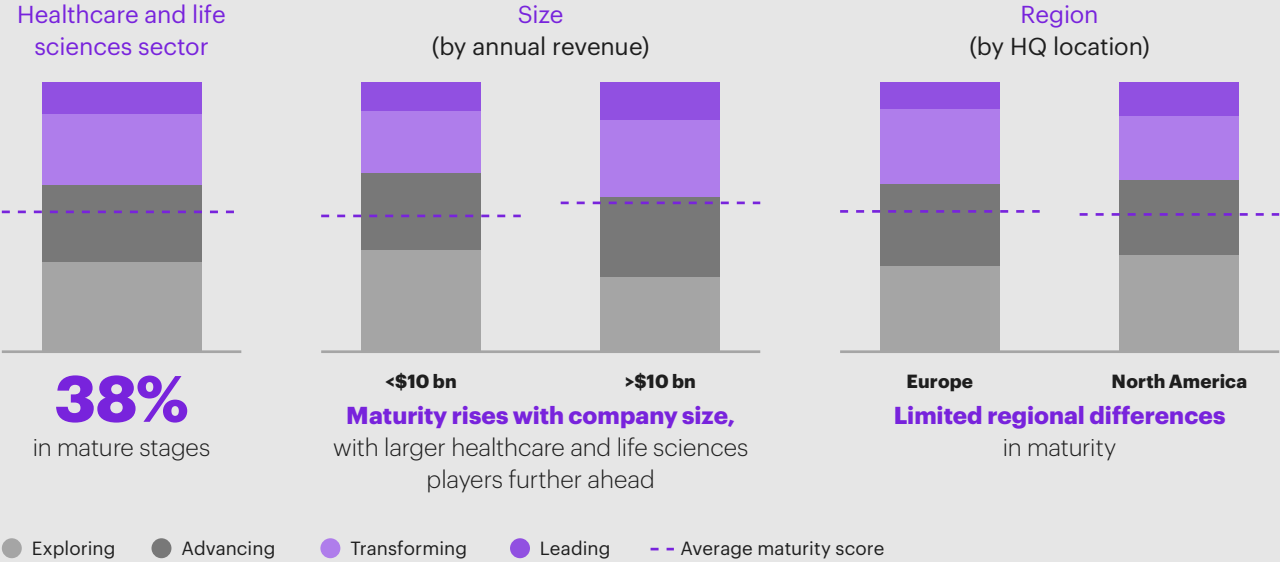
Across the ecosystem, just 38 percent of organizations have reached mature stages of action.

Figure 1
Half of organizations have made active efforts to close the gender health gap



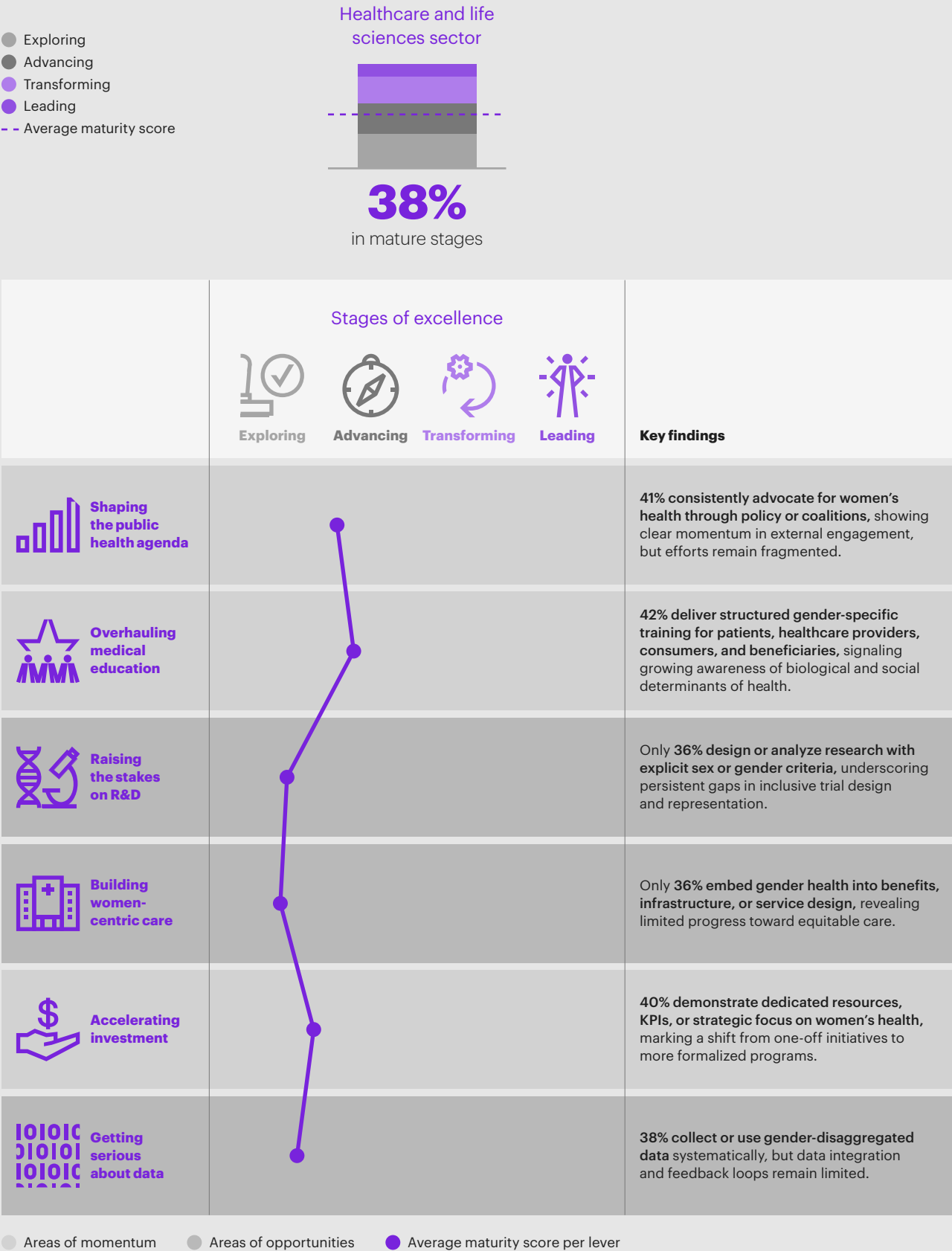
Source: Kearney analysis

Figure 2
Healthcare and life sciences organizations remain in the early stages overall, but larger organizations are showing stronger maturity while regional differences are limited



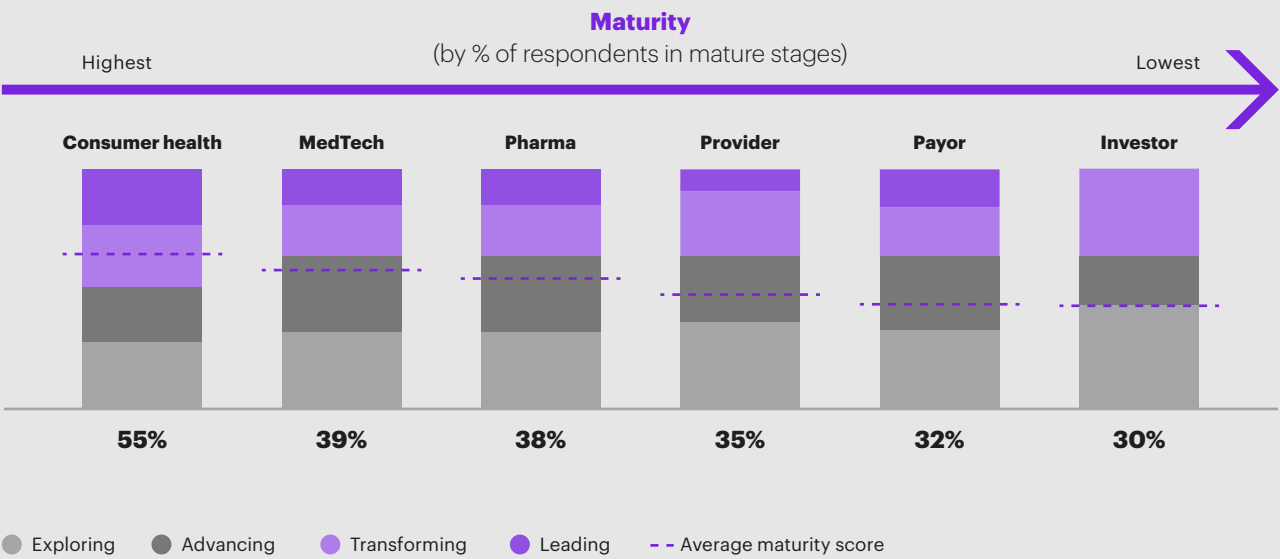
Source: Kearney analysis

Figure 3
Only 38% of healthcare and life sciences organizations show maturity in advancing women’s health, with progress strongest in education and advocacy but weakest in research and care



Source: Kearney analysis

Figure 4
Most healthcare and life sciences indexes indicate early-stage efforts in advancing women’s health, while the consumer health index shows slightly more mature progress



Source: Kearney analysis

Consumer health:

Leading the way through data, design, and direct connection to women

Consumer health is the most advanced segment, with 55 percent of companies in mature stages (*transforming or leading*), the highest across all sectors (see figure 5 on page 10). Progress is strongest in data, R&D, and advocacy, where most companies are integrating sex-disaggregated data and gender insights into product design, marketing, and engagement strategies. About 70 percent use gender data to inform development, and 60 percent invest in digital and personalized approaches to women’s health. However, progress on affordability, access, and policy influence remains limited. Few companies are translating their commercial reach into system-level change by engaging with governments and payors to embed women’s health into broader health frameworks.

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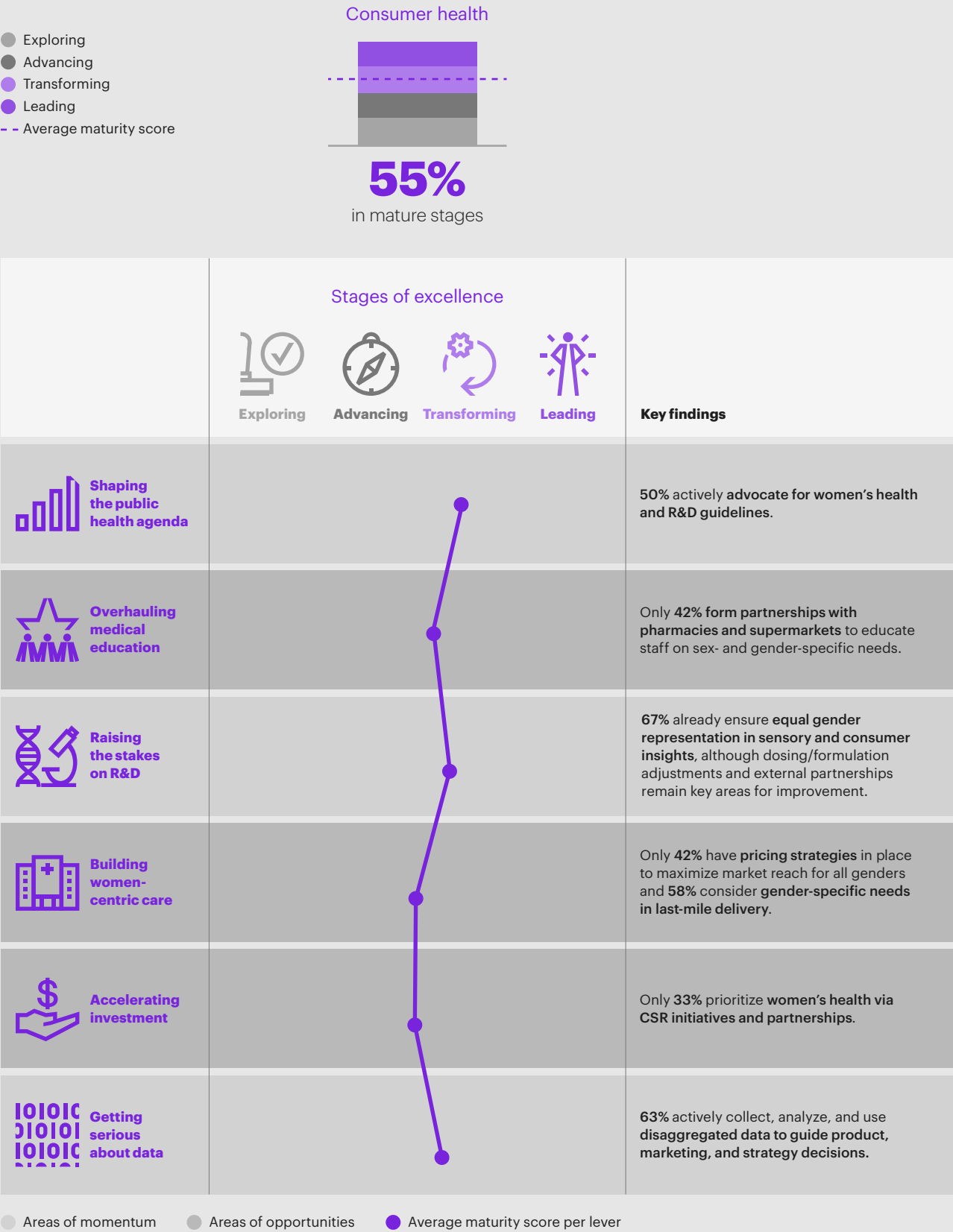
Case study

Essity shows how the sector can bridge that gap between innovation and policy. Despite more than [two billion people menstruating each month](#), their needs are rarely reflected in public policy. To address this, Essity worked with Stanford University and the Karolinska Institute to create the [Policy Roadmap for Closing the Menstrual Health Gap](#), which is endorsed by UNFPA and other global partners. The road map promotes a “[menstrual health in all policies](#)” approach, encouraging policymakers to integrate menstrual health in education, labor, and health systems. Essity pairs this policy work with practical innovation in menstrual products and education programs, showing how a global consumer health company can use its expertise and reach to improve access, dignity, and long-term social progress.

Case study

Bayer Consumer Health demonstrates how companies can turn product innovation and commercial scale into meaningful impact for women’s health. Its portfolio spans prenatal nutrition, intimate health, menopause, and period pain relief, with products such as [Elevit®](#), [Canesten®](#), and [Midol®](#). Through the [Nutrient Gap Initiative](#), Bayer and Vitamin Angels have reached more than 20 million women and babies with essential micronutrients. Partnerships with WomenHeart and reach52 extend care and education to more than 700,000 people, while the Canesten Vagina Academy uses science-based content to challenge stigma and reach more than 100 million online. By linking brand leadership with data, education, and access, Bayer has accelerated advancements in women’s health across life stages and settings.

Figure 5
Consumer health companies are in the *transforming* stage, with major progress in data and R&D, but trailing on women-centric care and investment



Source: Kearney analysis

MedTech:

Strong on education and awareness, but inclusion in validation and data remains limited

MedTech follows consumer health, with 39 percent of companies in mature stages (see figure 6 on page 12). Strength lies in education and awareness, where around 50 percent of organizations have launched women’s health programs for healthcare professionals or patients. Yet fewer than 30 percent collect sex-disaggregated data, and just above 30 percent include sex-specific sub-populations (for example, pregnant and lactating women) in trial populations where applicable. Because of that, the sector’s innovation often ends before it reaches inclusive clinical validation. The Index identifies clear priorities: embed sex- and gender-specific endpoints, include pregnant and lactating women in studies, and ensure transparent reporting of results.

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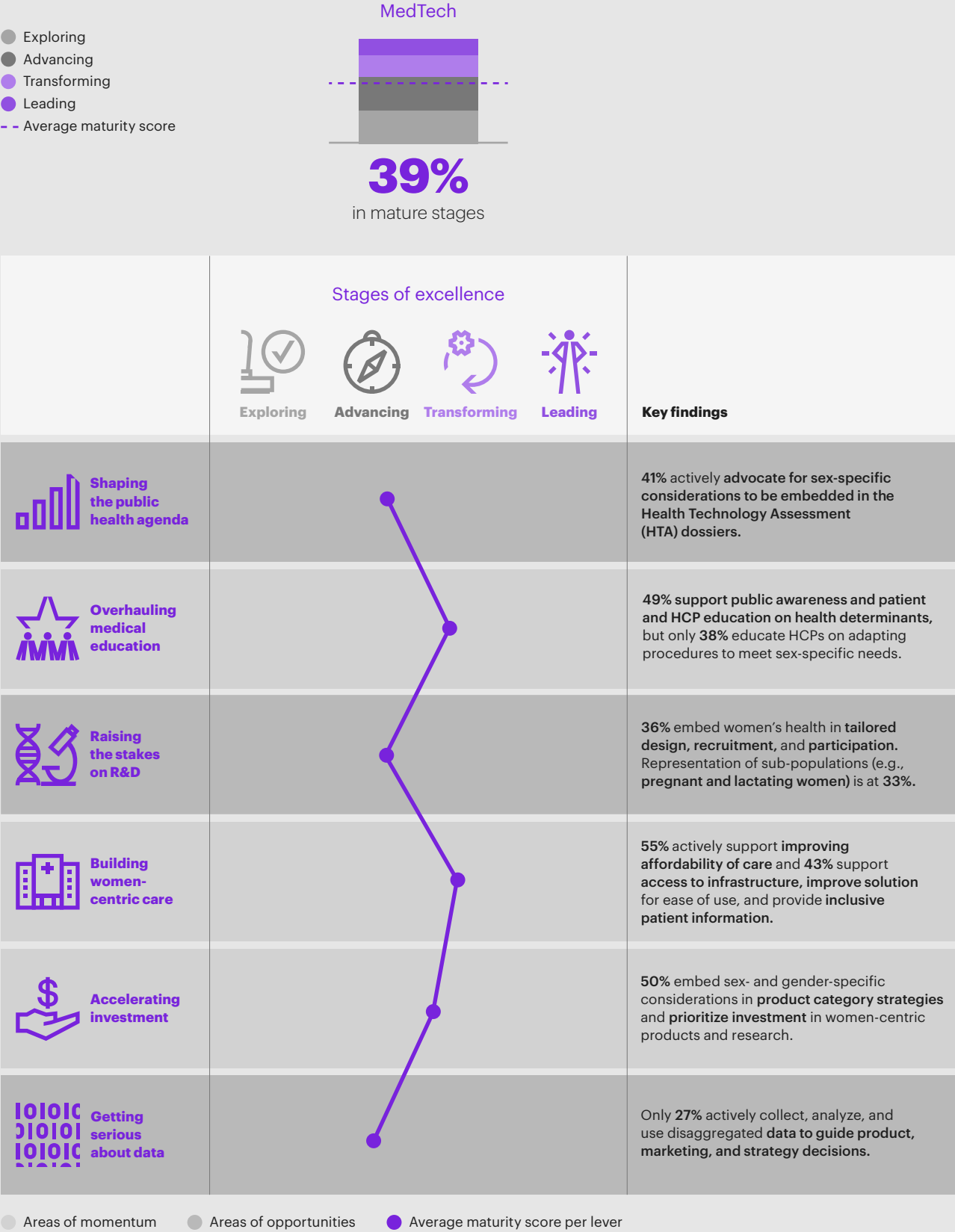
Case study

Roche Diagnostics demonstrates how inclusive design can shift outcomes. Cervical cancer, driven largely by persistent HPV infections, sees more than 600,000 new cases and causes 340,000 deaths annually. Persistent HPV infections are the primary driver, with incidence highest in regions lacking access to screening. High-income countries have already reduced cervical cancer incidence by up to 80 percent in some settings, especially through early investments in screening. Roche’s HPV self-collection solution empowers women to protect themselves against cervical cancer by collecting their own vaginal sample, overcoming cultural and logistical barriers that limit screening. Supporting the WHO 90–70–90 elimination strategy, Roche’s approach shows how data-driven MedTech innovation can expand access, reduce inequities, and accelerate progress toward disease elimination.

Case study

Breast cancer claims more than 670,000 lives globally each year, despite early detection offering survival rates of up to 91 percent for all SEER stages combined. To close persistent gaps in women’s health awareness and access, **Siemens Healthineers** has launched several initiatives. A five-year partnership with City Cancer Challenge (C/CAN) advances equitable cancer care through earlier diagnosis and improved outcomes. In Indonesia, Siloam Hospitals Group, in partnership with Siemens Healthineers, provided free breast cancer mammography screening using an AI-based application for reading mammograms, enabling the experts to provide an important second opinion and reducing the workload of radiologists. In collaboration with the Global Alliance for Women’s Health (GAWH), Siemens Healthineers is also strengthening gender-focused public outreach. Together, these initiatives demonstrate how technology and partnership can drive earlier detection, improve equity, and empower women to take control of their health.

Figure 6
MedTech companies are in the *advancing* stage, with some progress in education and women-centric care, with slower progress in data and R&D



Source: Kearney analysis

Pharma:

Advancing policy and education, but lagging in inclusive research and care design

Pharmaceutical companies show similar maturity, with 38 percent in advanced stages. Progress has been strongest in policy and professional education, but gaps remain in research and care delivery (see figure 7 on page 14). Only 31 percent of companies take substantial efforts to support affordability, accessibility, and inclusivity, and fewer than 15 percent adjust dosing and formulation to reflect biological differences. The sector's next frontier is embedding women's health across the R&D life cycle, moving beyond awareness to measurable inclusion and accountability in clinical science.

Case study

UCB advances inclusion by addressing one of the industry's most persistent gaps: the exclusion of pregnant and breastfeeding women from trials. Women of childbearing age with chronic conditions face persistent evidence gaps because pregnant and breastfeeding individuals make up **less than 0.4 percent of trial participants** in the EU. For more than a decade, UCB has prioritized research in these populations, developing protocols that allow women to remain in studies during pregnancy. Since 2023, it has also contributed to the ICH E21 Expert Working Group, supporting global guidelines for the inclusion of pregnant and breastfeeding women in trials. These efforts create more representative data and improve care for women with chronic conditions.

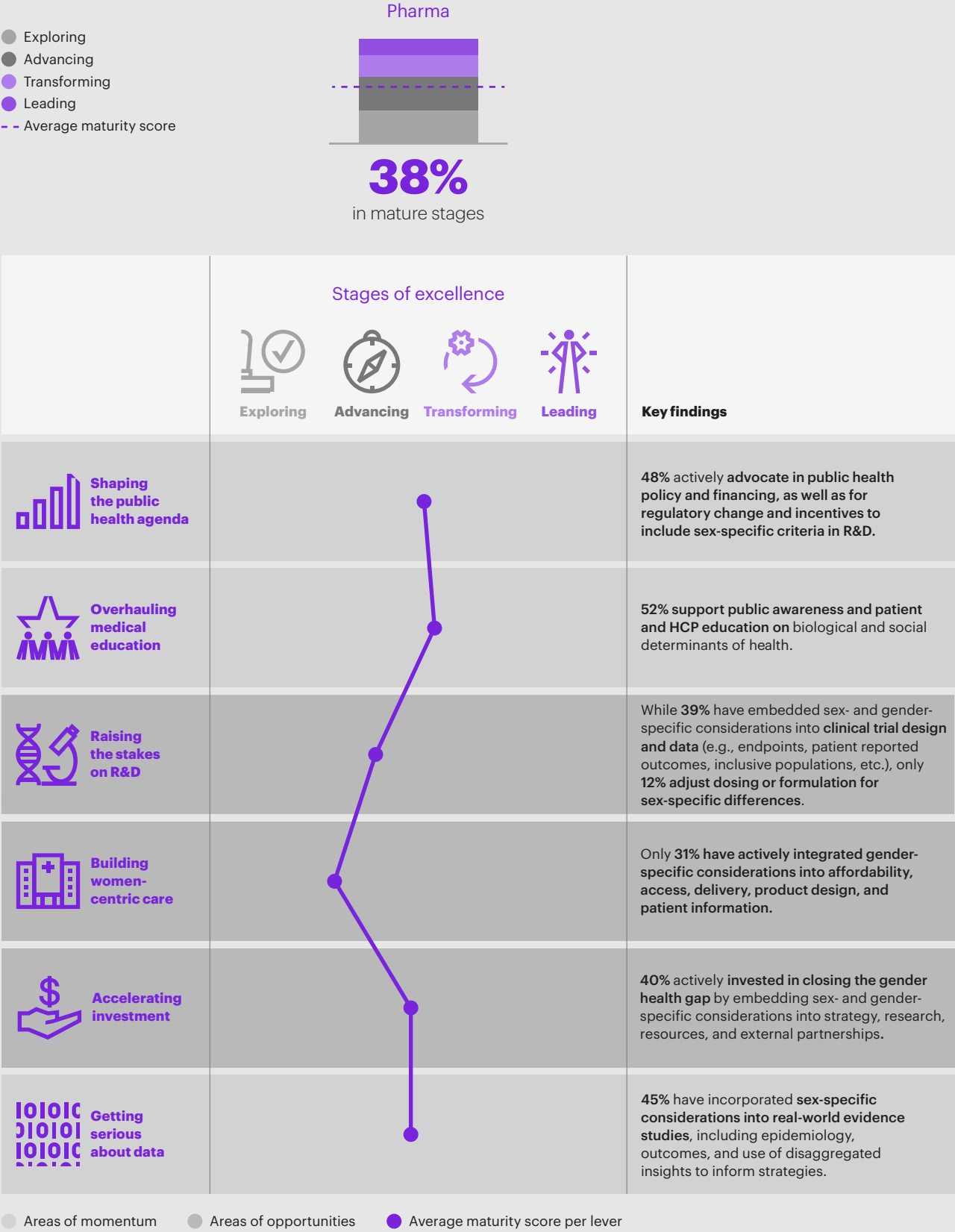
Case study

Ferring Pharmaceuticals's Project Safe Birth illustrates this shift. Post-partum hemorrhage causes **more than 20 percent of maternal deaths globally**, while infertility affects one in six people. Ferring's heat-stable carbetocin, now **listed as an essential medicine by the WHO**, enables safe delivery in low-resource settings by eliminating cold-chain dependency. In fertility, the company has invested in tools and resources to support organizations and individuals with the mental health and workplace challenges of family building, recognizing that outcomes extend beyond clinical interventions alone. Through its participation in coalitions such as the WEF Global Activators Network, Ferring links scientific innovation with equitable access, advancing maternal health and reproductive equity.

Case study

Economic and logistical barriers frequently limit participation among marginalized and underrepresented groups, reducing the rigor and generalizability of clinical research. **Roche/Genentech** implemented Enhanced Patient Support Services (EPSS), embedded in all US study contracts since 2022 and expanding globally, to cover transportation, equipment, and financial assistance. In the evERA Breast Cancer study, participants received per-visit stipends, concierge travel support, lodging reimbursements, and access to community-oncology patient navigation and a Social Determinants of Health pilot to address structural barriers. These efforts enabled equitable participation and delivered the highest US enrollment and greatest representation of Black/African American and Hispanic/Latino patients in any Roche/Genentech breast cancer trial to date.

Figure 7
Pharma companies are in the *advancing* stage, showing most progress in policy and education, but lagging in women-centric care and R&D



Source: Kearney analysis

Providers:

Investing in infrastructure and awareness, but integration into care pathways remains limited

Providers sit in the middle, with 35 percent in mature stages. They show solid progress in policy, education, and infrastructure, but only around one in three structurally embed sex- and gender-specific priorities in strategies or initiatives, and roughly the same ensure clinical trial populations reflect disease epidemiology (see figure 8 on page 16). Most still lack integrated data systems to monitor differences in outcomes between men and women. Sustained improvement will require integration of gender considerations into care pathways and adoption of measurable equity indicators to guide improvement.

Case study

Sehat Kahani offers a clear approach to sex- and gender-responsive care pathway design and delivery. Across South Asia, patients face barriers ranging from cost and distance to cultural restrictions, while many trained female doctors remain underutilized. In Pakistan alone, **35 percent of female medical graduates** are unemployed or out of practice. Sehat Kahani’s hybrid telemedicine platform addresses both challenges. With 65+ nurse-led e-clinics in rural areas and a mobile app for urban users, the platform provides timely, affordable, high-quality care. Women, in particular, benefit from confidential, culturally sensitive consultations with female doctors, encouraging care-seeking for sensitive health needs. The model also reintegrates female doctors into the workforce, creating a more inclusive healthcare system that delivers community-wide benefits.

Payors:

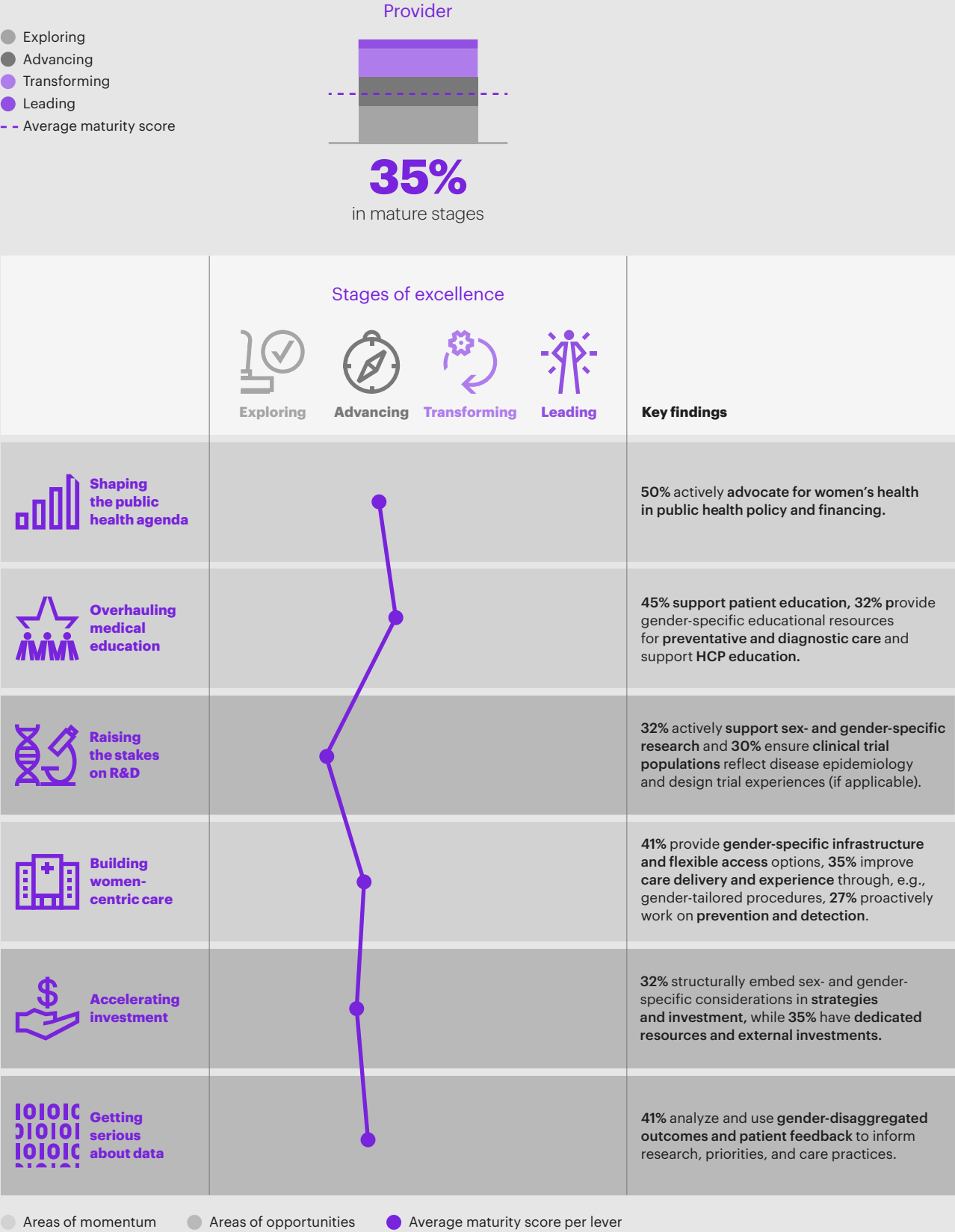
Driving awareness through policy, yet slow to translate intent into equitable access

Payors are at an earlier stage, with 32 percent in mature categories. They lead in policy and education, where about 40 percent have adopted gender-sensitive frameworks, but lag in benefit design and reimbursement models that would translate intent into equitable access (see figure 9 on page 17). Fewer than 25 percent cover trial participation costs or fund sex-specific research. Fewer than one in five payors integrate sex- and gender-specific considerations into benefit design, formularies, and cost-sharing programs, underscoring limited progress in aligning affordability and access with women’s health needs. Closing these gaps will require aligning payment systems, coverage policies, and incentives around women’s health needs.

Case study

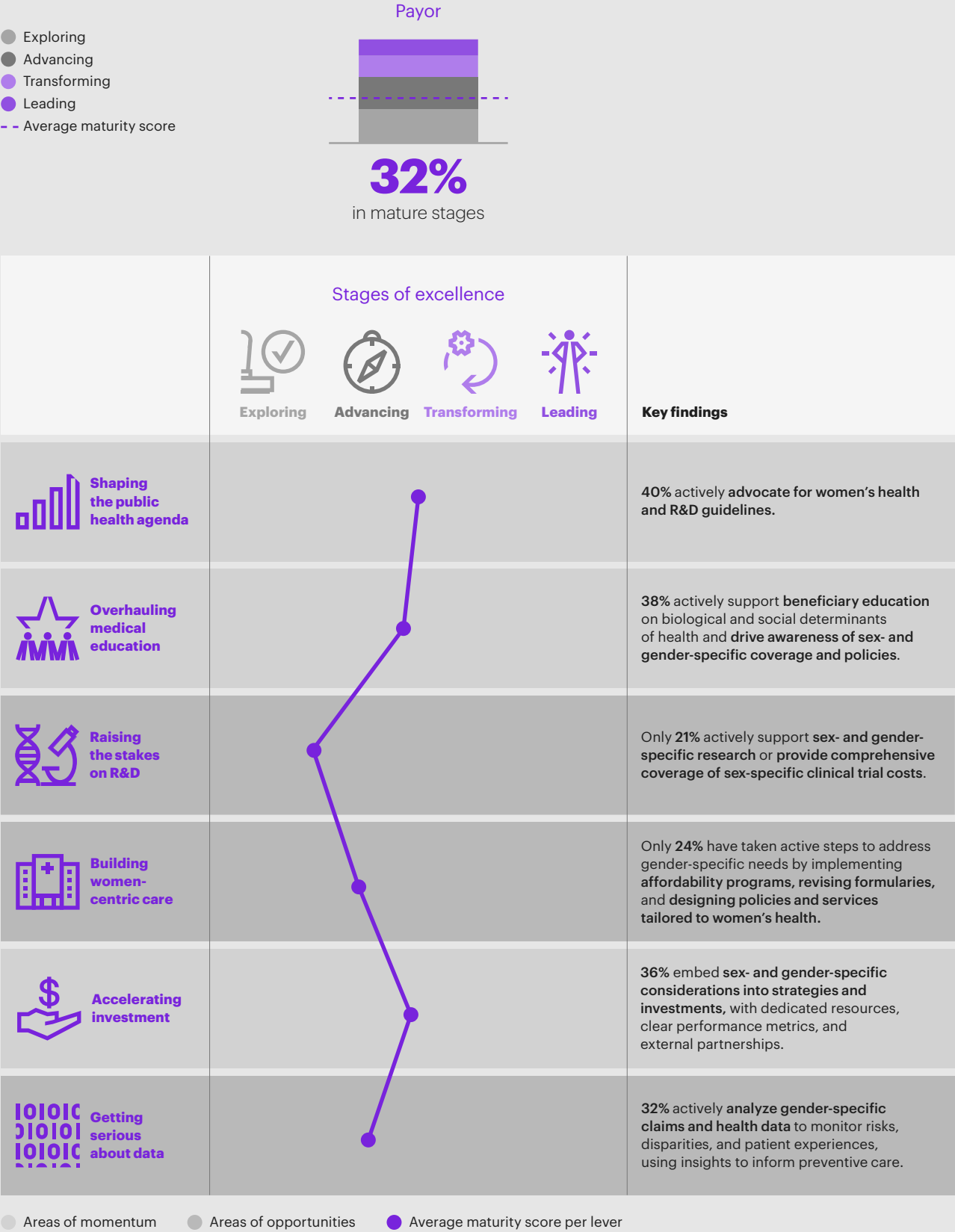
Kaiser Permanente illustrates how payors can drive equity through targeted investment. Uterine fibroids affect **more than 70 percent of women by age 50**, often causing pain, heavy bleeding, and fertility challenges. Traditional treatments are invasive and can result in long recovery times or loss of fertility. Gynesonics developed the Sonata Treatment, a uterus-preserving, incisionless procedure that treats fibroids from within the uterus. To scale access, Kaiser Permanente Ventures invested \$6.5 million to support reimbursement and commercialization, helping expand operations and validate market demand. By aligning investment with coverage innovation, Kaiser Permanente leads the way in crafting innovative reimbursement pathways that accelerate adoption of equitable, women-centered care.

Figure 8
Providers are mostly in the *advancing* stage, showing progress in policy, education, and infrastructure, but still lagging in research, care delivery, and investment maturity



Source: Kearney analysis

Figure 9
Payors are in the advancing stage, with relative progress in policy and education, and lagging in women-centric care and in supporting sex-specific research and trial cost coverage



Source: Kearney analysis

Investors:

Emerging as catalysts for change, but gender-lens capital remains underutilized

Investors remain the least mature, with only 30 percent in advanced stages. While interest in women's health is increasing, fewer than 25 percent of funds apply a gender-lens strategy during investment due diligence, and only one in seven incorporate gender considerations into value creation and portfolio management (see figure 10 on page 19). Women represent less than 20 percent of investors, perpetuating blind spots in decision-making. Still, the investment case is strong: women's health combines large unmet need, high innovation potential, and low competition. Progress will depend on investors embedding gender criteria across diligence, portfolio management, and impact reporting, supported by incentives that reward equity-focused capital.

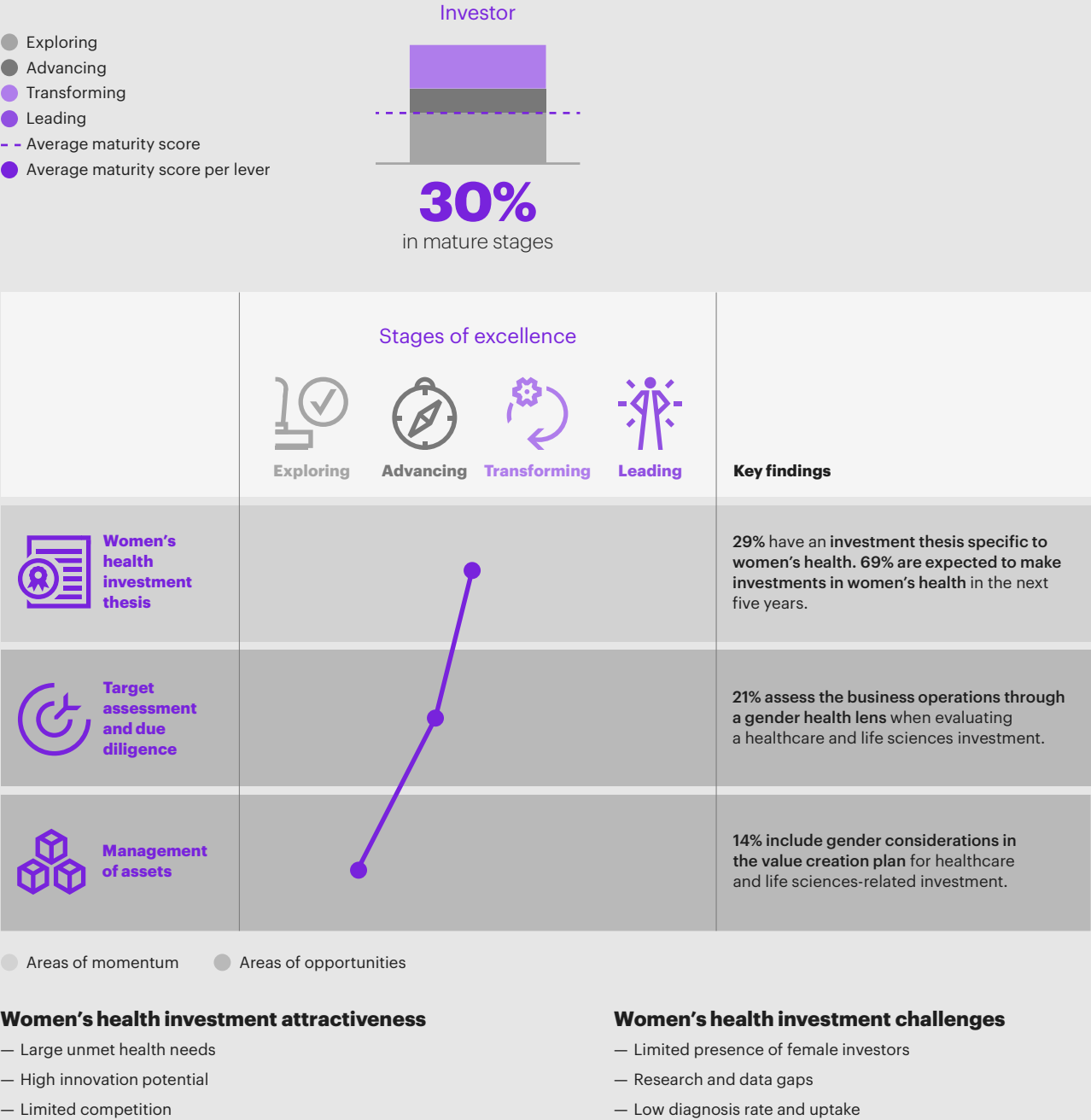
Case study

Philanthropic investors are already demonstrating how catalytic capital can accelerate progress. In 2025, the [Gates Foundation](#) announced a [\\$2.5 billion commitment through 2030](#) to advance research and development in neglected areas of women's health, including preeclampsia, endometriosis, menopause, and contraception, representing one of the largest philanthropic investments ever made in the field. Meanwhile, [Wellcome Leap](#) has launched [three \\$50 million programs in women's health](#) focused on gestational development, dementia, and heavy menstrual bleeding. In addition, in September 2025, Wellcome Leap and [Pivotal](#) committed [\\$100 million](#) to expand women's health research, bringing Leap's total investment to \$250 million. Together, these initiatives exemplify how mission-driven investors can de-risk innovation, expand access, and build a more equitable global health ecosystem.

**Investors
remain the least
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Figure 10

Investors are just entering the *advancing* stage, with growing interest in women’s health but limited integration of gender considerations into theses, due diligence, and value creation



Call to action

A system-wide call to action

Closing the women's health gap now requires a decisive shift from intent to execution. The [w]Health Healthcare and Life Sciences Index makes clear that while awareness is rising, accountability remains uneven and progress fragmented. Leaders across the healthcare ecosystem must act within their sphere of influence while aligning around shared priorities that enable system-wide change.

Embed sex and gender into core decision-making, not side initiatives. Across sectors, sex- and gender-specific considerations must be integrated into core operations, not treated as add-ons. For pharma and MedTech, this means designing R&D pipelines and clinical trials that reflect real-world epidemiology, include underrepresented populations, and translate biological differences into dosing, formulation, and evidence generation. For consumer health, it means moving beyond insight-led design to influence policy, access, and affordability at scale. For providers and payors, it requires embedding gender considerations into care pathways, benefits, and coverage decisions so equity is measured in outcomes, not intentions.

Make data actionable and leadership accountable.

Progress will stall without stronger data discipline and executive ownership. Organizations should systematically collect, analyze, and apply sex- and gender-disaggregated data across research, development, care delivery, and outcomes. Crucially, this data must inform strategy, capital allocation, and performance management. Setting clear KPIs tied to women's health outcomes and linking them to leadership incentives is no longer optional. Transparent reporting and benchmarking through tools like the [w] Health Index are essential to move from isolated efforts to sustained, measurable progress.

Align access, investment, and incentives to scale impact. Equitable access remains one of the largest gaps across the ecosystem. Payors and providers must translate policy commitments into benefit design, reimbursement models, and delivery infrastructure that meet women's needs across life stages and geographies. Investors, meanwhile, should treat women's health as a strategic growth opportunity, embedding gender-lens criteria into diligence, portfolio management, and value creation. Policymakers can accelerate progress by aligning regulation, incentives, and procurement to reward inclusive research, equitable access, and transparent reporting. Without this alignment, innovation will continue to outpace adoption.

The opportunity is clear. Organizations that embed gender equity into strategy, operations, and investment will not only close persistent health gaps but also unlock scientific, economic, and societal value. The [w]Health Index provides a common language and a practical road map for this next phase. The question for leaders is no longer whether to act, but how quickly they are prepared to lead.

What leaders should do in the next 12 months

(Sector-specific actions, aligned to shared system goals)

Consumer health

- Move beyond consumer insight to system impact by using data and scale to influence public policy, access, and affordability for women’s health products.
- Embed women’s health priorities into pricing, distribution, and last-mile delivery strategies, particularly for underserved populations.
- Use brand reach to normalize women’s health across the life course and challenge stigma at scale.

MedTech

- Ensure validation studies and real-world evidence generation reflect sex-specific use cases and outcomes, particularly for diagnostics and devices used across life stages.
- Standardize the collection and use of sex-disaggregated data across product development, regulatory submissions, and post-market surveillance.
- Partner with providers and payors to embed women-centric technologies into care pathways, not just pilots.

Pharma

- Redesign at least one priority R&D program to embed sex- and gender-specific endpoints from protocol design through evidence generation, not only post hoc analysis.
- Expand inclusion of underrepresented female sub-populations (including pregnant and lactating women where appropriate) and translate findings into dosing, formulation, and labeling decisions.
- Tie women’s health KPIs to portfolio governance and senior leadership accountability.

Providers

- Integrate sex- and gender-specific considerations into clinical pathways, quality metrics, and outcome monitoring across priority conditions.
- Use data to identify and close gaps in diagnosis, treatment, and outcomes between women and men.
- Invest in workforce training and care models that reflect women’s lived experiences and health needs.

Payors

- Translate policy intent into benefit design by aligning coverage, reimbursement, and cost-sharing with women’s health needs across life stages.
- Pilot reimbursement models that support inclusive research participation, prevention, and early intervention.
- Use claims and outcomes data to proactively identify inequities and redesign access accordingly.

Investors

- Treat women’s health as a strategic growth theme by embedding gender-lens criteria into diligence, investment theses, and value-creation plans.
- Require portfolio companies to report on sex- and gender-specific metrics and outcomes.
- Use catalytic capital and partnerships to de-risk innovation in underfunded women’s health areas.

Conclusion

Women's health is no longer a peripheral issue. It is a defining measure of health system performance, scientific progress, and economic resilience. The evidence presented in the [w]Health Healthcare and Life Sciences Index shows clear momentum, but it also makes one thing unmistakable: progress remains uneven, fragmented, and too often disconnected from core business and policy decisions.

The next phase requires leadership that moves beyond awareness to accountability. Closing the gender health gap demands that sex and gender are embedded as design principles across research and innovation, care delivery, data systems, and investment decisions. It requires leaders to treat equity not as a standalone initiative, but as a driver of better outcomes, stronger markets, and more resilient societies.

The [w]Health Index provides the first cross-sector benchmark to support this shift. By establishing a shared baseline, defining clear levers for action, and enabling transparent measurement over time, it offers organizations a practical road map to translate commitment into results. Used consistently, it can help align efforts across sectors and turn fragmented initiatives into coordinated system change.

The inequities in women's health are no longer invisible. The data are clear. The tools exist. The path forward is defined. What is required now is sustained leadership and execution. Organizations that act decisively will not only close long-standing health gaps, they will help redefine what effective, inclusive, and future-ready healthcare looks like.

Closing the gender health gap demands that sex and gender are embedded as design principles across research and innovation, care delivery, data systems, and investment decisions.

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About Equity 2030 Alliance

The Equity 2030 Alliance is a global effort to normalize gender equity in science, technology and financing solutions by introducing data-driven arguments and strategies. Led by the United Nations Population Fund (UNFPA), the Alliance unites experts from business, academia and government to accelerate joint actions leading to an inclusive, sustainable and gender-equitable future, where women's and girl's voices are heard, their needs are met and their potential is fulfilled.

unfpa.org/equity-2030-alliance

About HBA Think Tank

The HBA Think Tank is an award-winning, research-driven initiative that brings together senior leaders across the globe to shape the future of the healthcare workforce. With a focus on the Future of Work, the Think Tank identifies industry challenges such as wellness, talent readiness, digital transformation, and leadership advancement. Through global surveys, data-driven summits, and cross-industry collaboration, it generates innovative, evidence-based solutions that organizations can apply directly. By turning insights into action, the HBA Think Tank is driving measurable progress in workplace design, health outcomes, and sustainable workforce strategies across healthcare and life sciences.

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